ADR REPORTING FORM

B.Pharm / Pharmacovigilance – Step-by-Step Study Guide

Video: “How to fill ADR form? Step by Step Guide”

Important note: The linked YouTube page exposes its title but not a readable transcript here. This guide is therefore based on the official Indian Pharmacopoeia Commission (IPC) / Pharmacovigilance Programme of India (PvPI) healthcare-professional ADR form (Version 1.4), rather than a claim that every point below was spoken in the video.

1. Meaning and purpose

ADR (Adverse Drug Reaction): A harmful or unintended response suspected to be associated with the use of a medicine. ADR reporting helps detect safety signals, improve medicine safety, and support pharmacovigilance. A report records a suspicion; it does not by itself prove that the medicine caused the reaction.

2. Before filling the form

• Use the current official PvPI form for healthcare professionals.

• Collect accurate information from the patient record and the responsible healthcare professional.

• Record dates, doses, route, frequency, medicine names, reaction details, relevant history, and available investigations.

• Protect patient confidentiality. Use initials rather than writing unnecessary identifying details.

• Do not delay a report merely because some details are unknown; mark unknown or unavailable where appropriate.

3. Form-filling sequence

Section / field	What to enter
Report type	Tick Initial Case for the first report; Follow-up Case when sending additional information about an already reported case.
A. Patient information	Patient initials, age or date of birth, gender, and weight (kg), as available.
B. Suspected adverse reaction	Reaction/event start and stop dates; clear description; management provided; relevant investigations with dates.
Medical / medication history	Relevant allergies, pregnancy status where relevant, medical conditions, and hepatic/renal history, as applicable.
Seriousness and outcome	Tick the applicable seriousness criterion and record outcome: recovered, recovering, not recovered, fatal, recovered with sequelae, or unknown.
C. Suspected medication(s)	Brand/generic name, manufacturer, batch/lot and expiry if known, dose, route, frequency, therapy start/stop dates, indication, and causality assessment if available.
Action taken / reintroduction	Record what happened to the suspected medicine after the reaction. Document reappearance only if it actually occurred; never deliberately restart a medicine to test this.
Concomitant products	List other medicines, self-medication, and herbal products with dose, route, frequency, dates, and indication (excluding treatment given for the reaction).
D. Reporter details	Reporter name/address, contact details, occupation, signature, and report date.

4. Important terms to understand

Indication: The medical condition or reason for which the medicine was prescribed.

Dechallenge: The reaction’s course after the suspected medicine is stopped or its dose is reduced, when this occurs as part of appropriate clinical care.

Rechallenge: The reaction’s recurrence after the medicine is restarted. This is recorded if it happened; it must not be deliberately attempted just to confirm an ADR.

Concomitant medication: Another medicine or health product taken during the same period.

Serious ADR: An event meeting seriousness criteria such as death, life-threatening condition, hospitalization/prolonged hospitalization, significant disability, congenital anomaly, or another medically important event. Seriousness is not the same as intensity.

Causality assessment: A structured evaluation of how likely the medicine is related to the reaction; examples include WHO-UMC and Naranjo methods.

5. Final checking checklist

■ Mandatory fields are completed as far as information is available.

■ Medicine name, dose, route, frequency, and dates are consistent.

■ Reaction description is factual, clear, and chronological.

■ Relevant investigations and other medicines are included.

■ Seriousness and outcome are selected accurately.

■ Reporter details and report date are present.

■ Patient confidentiality is maintained.

6. What you should learn / practical task

For your B.Pharm practical or assignment: (1) Download and identify the official PvPI ADR form; (2) learn the four main sections A–D; (3) practise entering information using a clearly labelled fictional classroom case; (4) explain indication, seriousness, outcome, dechallenge, rechallenge, concomitant medicines, and causality assessment; (5) check the completed form for missing or inconsistent information. Do not submit fictional information as a real ADR report.

7. Official references

• CDSCO / PvPI – Suspected Adverse Drug Reaction Reporting Form for Healthcare Professionals, Version 1.4: https://www.cdsco.gov.in/opencms/export/sites/CDSCO_WEB/Pdf-documents/Consumer_Section_PDFs/ADRRF_2.pdf

• Indian Pharmacopoeia Commission – ADR Reporting Forms: https://ipc.gov.in/mandates/pvpi/adr-reporting/8-category-en/865-adr-reporting-forms.html

Study reminder: Report suspected reactions through the appropriate PvPI / healthcare-professional process. Clinical assessment and treatment decisions belong to qualified healthcare professionals.